

Operations Research for Health Care

Lesson 7/15

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Outline of this lecture

Hierarchical health care location problems

Analytical and improved 3-hierarchical system

Hierarchical with referral of patients

Intermediate facility location

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Review of hierarchical health care location [Ortiz-Astorquiza et al., 2018]

Multiple layer configuration: Network with **k-levels**: nodes represent demand site and facilities. Patients on level 0, primary-care on level 1, ... highest level on level k. Classification:

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- ▶ **Flow pattern:** single-flow, multi-flow, bidirectional-flow. (referral system).
- ▶ **Service:** successively inclusive and exclusive (nested and non-nested).
- ▶ **Spatial configuration:** Coherent or non-coherent system.
- ▶ **Objectives:** minimizing total transportation costs/distances (*median*), cost for opening (*fixed charge*), maximizing coverage (*maximal covering*), other.

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Given a set of patients that demand one or k services and a set of facilities associated with k services, select a set of facilities to open so that each patient receives the requested service(s), while optimizing a criteria.

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Analytical 3-hierarchical system [Calvo and Marks, 1973]

Motivation:

- ▶ Locational criteria: hierarchy of social, economic and political attributes.
- ▶ Hospitals, health centers, nursing homes, and private clinics that are planned, organized, and operated independently of each other. **The result has been a large duplication of services, inefficient utilization of resources, and imbalance in facility locations.**
- ▶ The critical issue: *where should the facilities be located in a region in order to satisfy the health care requirements of its population and comply with the social, economic, and political criteria relevant to the region?*

Analytical 3-hierarchical system [Calvo and Marks, 1973]

The problem:

- ▶ **Regional hospital** (specialized treatment), **district hospitals** (intermediate care, *lacking the specialized treatment of regional facility*), **rural hospitals** (outpatient care and general inpatient care). There is intensive communication and *patient referral* between them. It is a **successively inclusive** property.

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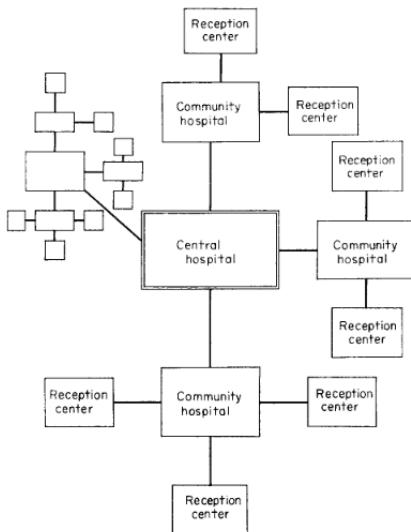
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- ▶ Within the **hospital**, the physicians' offices are to be located, with the necessary paramedical services and the appropriate communication devices and technology sources. The **community hospitals** offer essentially the same services, but of lesser degree, not offering the specialized services.

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- ▶ The **reception centers** will provide the “*entry points*” to the entire system for the population of the region. Principally, they will be staffed with *paramedical personnel and public health nurses providing auxiliary treatment, checkups and screening tests, and with social workers providing assistance with the community's social problems*. These centers will provide the public with **greater accessibility to health care services**.

Analytical 3-hierarchical system [Calvo and Marks, 1973]



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Measure of **effectiveness**. Geographical, financial accessibility, and effectiveness in meeting the demand and needs of the region. Three sectors of society are involved:

- ▶ **The user:** patient population (*time and cost to travel, service cost directly or indirectly, through taxes or insurances*);

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- ▶ **The community:** resident population and local businesses, socio-economic impacts. *Traffic, environment, political considerations and platforms of debate.*

Optimization Parameters: The **region** is disaggregated into n *planning areas*, each composed of s *population groups*. E.g., population **group** k in community i ($i = 1, 2, \dots, n$) will demand order of **service** k ($k = 1, \dots, s$). The allocation of demand should be conducted so as to satisfy the health care needs of the region and optimize the relevant planning criteria.

Let a_{ik} be the number of people in group k residing in area i and requiring order of services k . The total population in area i is

$$a_i = \sum_{k \in 1..s} a_{ik}, \quad \forall i \in 1..n$$

Analytical 3-hierarchical system [Calvo and Marks, 1973]

All population groups within an area have to be fully assigned to the corresponding facility-types.

$$\sum_{k \in 1}^{k^*} \sum_{j \in 1}^n x_{ijk} = 1 \quad \forall \quad x_{ijk^*} > 0 \quad (2)$$

$$\sum_{j \in 1..n} x_{ijk} = 1, \quad \forall \quad k^* < k < s, k^* \in 1..s, i \in 1.., n \quad (3)$$

Constraints (2) and (3) depend on *variable* K_i , obtained only after the facilities are located. A *chicken-and-egg* dilemma (impossible) to resolve [Tien et al., 1983].

a) (2) is replaced by (8), which assures that individuals requiring different types of services cannot obtain these services from the same facility, and b) the range in (9) is not dependent on a K_h as is the case in (3) [Tien et al., 1983].

$$\sum_{k \in 1..s} x_{ijk} \leq 1 \quad \forall \quad i \in 1..n, j \in 1..n \quad (8)$$

Analytical 3-hierarchical system [Calvo and Marks, 1973]

Assignment to only those areas which assign to themselves, for each order k facility. This avoids the possibility of a population group a_{ik} being assigned to area j , where there is no self assignment, that is no facility located there

$$x_{jjk} > x_{ikj}, \quad \forall \quad i = 1..n, j \in 1..n, k \in 1..s \quad (4)$$

A specific number of order k facilities (for $k = 1, 2, \dots, s$) that may assign areas to themselves.

$$\sum_{i \in n} x_{iik} = m_k, \quad \forall \quad k = 1..s \quad (5)$$

$$x_{ijk} = \{0, 1\} \quad \forall \quad i \in 1..n, j \in 1..n, k = 1..s \quad (6)$$

As an alternative to a fixed number of facilities, the optimization may be carried out under a specified budget or investment level.

Analytical 3-hierarchical system [Calvo and Marks, 1973]

USER:

- ▶ minimize Weighted Distance: $\sum_{i \in 1..n} \sum_{j \in 1..n} d_{ij} \sum_{k \in 1..s} a_{ik} x_{ijk}$
- ▶ minimize Total Cost: $\sum_{i \in 1..n} \sum_{j \in 1..n} c_{ij} \sum_{k \in 1..s} a_{ik} x_{ijk}$

SERVICE:

- ▶ F_{jk} : (fixed cost for building facility k in area j)
- ▶ V_{jk} : (service and expansion cost for operating facility k in area j)
- ▶ S_{jk} : size of k facility, which depends on demand, i.e,
$$S_{jk} = \sum_{i \in 1..n} a_{ik} x_{ijk}, \quad \forall j, k$$
- ▶ Facility Cost = $\sum_{j \in 1..n} \sum_{k \in 1..s} p_{jk} * F_{jk} * x_{ijk} + V_{jk} \sum_{i \in 1..n} a_{ik} * x_{ijk}$
- ▶ $l_k \leq \sum_{i \in 1..n} a_{ik} * x_{ijk} \leq u_k$ (lower and upper bound of capacity)

Objective Function:

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- ▶ $l_k \leq \sum_{i \in 1..n} a_{ik} * x_{ijk} \leq u_k$ (lower and upper bound of capacity)

Objective Function:

$$\text{minimize } \sum_{i \in 1..n} \sum_{j \in 1..n} c_{ij} \sum_{k \in 1..s} a_{ik} x_{ijk} + \sum_{j \in 1..n} \sum_{k \in 1..s} p_{jk} F_{jk} x_{ijk} + V_{jk} \sum_{i \in 1..n} a_{ik} x_{ijk} \quad (1)$$

Analytical 3-hierarchical system [Calvo and Marks, 1973]

LOCATION PLANNING PROCESS: it is necessary to place these models in a general planning framework (see fig 4 for discussion).

1. Define location problem

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2. Define locational social, political and economical criteria
3. Optimize and conduct sensitivity analysis
4. Evaluate "efficient solution". Satisfied? End, Else return to (2)

Analytical 3-hierarchical system [Calvo and Marks, 1973]

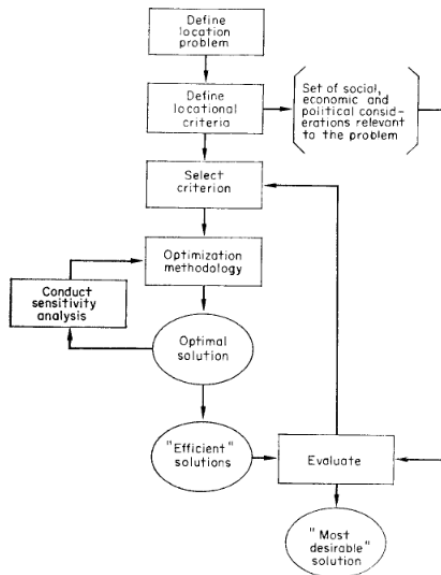


FIG. 4. Health facility location planning process.

Improved 3-hierarchical system [Tien et al., 1983]

$$\min. \sum_{i \in 1..n} \sum_{j \in 1..n} d_{ij} \sum_{k \in 1..s} a_{ik} * x_{ijk} \quad (13)$$

$$\text{s.t.: } \sum_{j \in 1..n} x_{ijk} = 1, \quad \forall i, k \quad (14)$$

$$x_{jjk} \geq x_{ijk}, \quad \forall i, j, k \quad (15)$$

$$\sum_{i \in 1..n} x_{iik} = \sum_{r=k..s} m_r \quad \forall k \quad (16)$$

$$x_{ijk} \in \{0, 1\} \quad \forall i \in I, j \in J, k \in K$$

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Hierarchical with referral of patients [Narula and Ogbu, 1979]

- ▶ **Motivation:** A patient who is not completely cured at a health center may be referred to a hospital or a medical center for treatment; and a patient who first goes to a hospital may be referred to a medical center, if necessary. A medical center is equipped to treat all cases. This system as successfully implemented in Cuba.
- ▶ Optimal service radius (distance) for each type of facility: net social benefit per capita = cost per capita (Schultz, 1970), but not realistic, because population is not homogeneous, and referral was not considered.
- ▶ [Calvo and Marks, 1973] assumed that the patients at each population center can be divided into $k(=3)$ groups according to their health care needs. They also did not allow for any referrals of the patients.
- ▶ **Offer:** In this paper, we allow the referral of $\theta(0 \leq \theta \leq 1)$ proportion of patients from a health center to a hospital.

I : The set of demand points.

K : The set of candidate locations for a level-1 PCF (e.g., clinics).

J : The set of candidate locations for a level-2 PCF (e.g., hospitals).

- ▶ $d1_{ik}$: The travel time from demand point $i \in I$ to a level-1 PCF in a candidate location $k \in K$.
- ▶ $d2_{kj}$: The travel time from a level-1 PCF in a candidate location $k \in K$ and a level-2 PCF in a candidate location $j \in J$.
- ▶ w_i : The population size, demand point at $i \in I$.
- ▶ p : The number of a level-1 PCF locations to be established.
- ▶ q : The number of a level-2 PCF locations to be established.
- ▶ $C1_k$: The capacity of a level-1 PCF in candidate location $k \in K$.
- ▶ $C2_j$: The capacity of a level-2 PCF in candidate location $j \in J$.
- ▶ θ_k : Proportion of level-1 PCF patients at candidate location $k \in K$ referred to a level-2 PCF.

- ▶ $x1_k \in \{0, 1\}$: 1, if a level-1 PCF is established at candidate location $k \in K$; 0 otherwise.
- ▶ $x2_j \in \{0, 1\}$: 1, if a level-2 PCF is established at candidate location $j \in J$; 0 otherwise.
- ▶ u_ik : The flow of patients between demand point $i \in I$ and a level-1 PCF at candidate location $k \in K$
- ▶ v_kj : The flow of patients between demand point $k \in K$ and a level-2 PCF at candidate location $j \in J$

$$\min. \sum_{i \in I} \sum_{k \in K} d1_{ik} u_{ik} + \sum_{k \in K} \sum_{j \in J} d2_{kj} v_{kj} \quad (28)$$

$$\text{s.t.: } \sum_{k \in K} u_{ik} = w_i \quad \forall i \in I \quad (29)$$

$$\sum_{j \in J} v_{kj} = \theta_k \sum_{i \in I} u_{ik} \quad \forall k \in K \quad (30)$$

$$\sum_{i \in I} u_{ik} \leq C1_k x1_k \quad \forall k \in K \quad (31)$$

$$\sum_{k \in K} v_{kj} \leq C2_j x2_j \quad \forall j \in J \quad (32)$$

$$\sum_{k \in K} x1_k = p \quad (33)$$

$$\sum_{j \in J} x2_j = q \quad (34)$$

$$x1_k \in \{0, 1\} \quad \forall k \in K$$

$$x2_j \in \{0, 1\} \quad \forall k \in K$$

$$u_{ik} \geq 0 \quad \forall i \in I, k \in K$$

$$v_{kj} \geq 0 \quad \forall k \in K, j \in J$$

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Scarce literature on health care.

- ▶ Publications are applied to warehouses, airport, and solid waste management
 - Kuehn, A. A., & Hamburger, M. J. (1963). A heuristic program for locating warehouses. *Management science*, 9(4), 643-666.
 - Helms, B. P., & Clark, R. M. (1971). Locational models for solid waste management. *Journal of the Urban Planning and Development Division*, 97(1), 1-13.
 - Kanafani, A. (1972). Location models for off-airport terminals. *Transportation Research*, 6(4), 371-379.
 - Mirchandani, P. B., & Odoni, A. R. (1979). Locating new passenger facilities on a transportation network. *Transportation Research Part B: Methodological*, 13(2), 113-122.
- ▶ **We propose:** an adaptation of [Kuehn and Hamburger, 1963] to health care.
- ▶ Primary health care (PHC) units are fixed, number of hospitals are also fixed. There are candidate locations of intermediate health care units (IHC).
- ▶ Patients adopt a minimum-cost flow from PHC to hospital using best IHC units.

Intermediate facility location [Kuehn and Hamburger, 1963]

H : Types of Patients (cronic or acute).

I : Primary health care units (FIXED).

J : Intermediate facility *candidates* location.

K : Hospital units (FIXED).

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- ▶ A_{hij} : Travel cost/patients $h \in H$, from PHC $i \in I$ to IHC $j \in J$.
- ▶ B_{hjk} : Travel cost/patients $h \in H$, from IHC $j \in J$ to Hospital $k \in K$.
- ▶ S_{hj} : Variable cost of IHC $j \in J$, per patients $h \in H$.
- ▶ F_j : Fixed cost per period for operating IHC $j \in J$.
- ▶ Q_{hk} : Patients $h \in H$, requiring service at Hospital $h \in H$.
- ▶ W_j : Capacity of IHC j .
- ▶ Y_{hi} : Capacity of PHC i meet health care requirements of patients h .

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- ▶ x_{hijk} : Patients h , flow from PHC i to IHC j to Hospital k .
- ▶ z_j : If IHC j is used (1) or not (0).

Intermediate facility location [Kuehn and Hamburger, 1963]

$$\min. \sum_h^H \sum_i^I \sum_j^J \sum_k^K (A_{hij} + B_{hjk})x_{hijk} + \sum_j^J F_j z_j + \sum_h^H \sum_i^I \sum_j^J \sum_k^K S_{hj}x_{hijk} \quad (1)$$

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$$\min. \sum_h^H \sum_i^I \sum_j^J \sum_k^K (A_{hij} + B_{hjk}) x_{hijk} + \sum_j^J F_j z_j + \sum_h^H \sum_i^I \sum_j^J \sum_k^K S_{hj} x_{hijk} \quad (1)$$

$$\text{s.t.: } \sum_{i \in I} \sum_{j \in J} x_{hijk} = Q_{hk} \quad \forall h \in H, k \in K \quad (2)$$

$$\sum_{j \in J} \sum_{k \in K} x_{hijk} \leq Y_{hi} \quad \forall h \in H, i \in I \quad (3)$$

$$\sum_{h \in H} \sum_{i \in I} \sum_{k \in K} x_{hijk} \leq W_j z_j \quad \forall j \in J \quad (4)$$

$$x_{hijk} \geq 0 \quad \forall h \in H, i \in I, j \in J, k \in K \quad (5)$$

$$z_j \in \{0, 1\} \quad \forall j \in J \quad (6)$$

Intermediate facility location - MathProgramming

```
set H;
```

```
set I;
```

```
set J;
```

```
set K;
```

```
param A{H,I,J};
```

```
param B{H,J,K};
```

```
param S{H,J};
```

```
param F{J};
```

```
param Q{H,K};
```

```
param W{J};
```

```
param Y{H,I};
```

```
var x{H,I,J,K}, >= 0;
```

```
var z{J}, >= 0, binary;
```


Intermediate facility location - MathProgramming

```
minimize Total_Costs:  sum{h in H, i in I, j in J, k in K}
(A[h,i,j]+B[h,j,k])*x[h,i,j,k] + sum{j in J}F[j]*z[j]
+ sum{h in H, i in I, j in J, k in K}S[h,j]*x[h,i,j,k];

s.t.  R1{h in H, k in K}:  sum{i in I, j in J}x[h,i,j,k] = Q[h,k];
s.t.  R2{h in H, i in I}:  sum{j in J, k in K}x[h,i,j,k] <= Y[h,i];
s.t.  R3{j in J}:  sum{h in H, i in I, k in K}x[h,i,j,k] <=
W[j]*z[j]; solve;
```

Good reports bring valuable information

```
printf:  "\n=====\\n";
printf:  "Intermediate Health Care Units\\n";
printf:  "=====\\n";
printf:  "Logist Cost:  $%6.2f\\n",
sum{h in H, i in I, j in J, k in K} (A[h,i,j]+B[h,j,k])*x[h,i,j,k];
printf:  "Fixed Cost:  $%6.2f\\n", sum{j in J}F[j]*z[j];
printf:  "Variable Cost:  $%6.2f\\n",
sum{h in H, i in I, j in J, k in K}S[h,j]*x[h,i,j,k];
printf:  "=====\\n";
printf:  "Total Cost:  $%6.2f\\n", Total_Costs;
printf:  "=====\\n";
printf:  "Hospital:  Requirement\\t Met\\n";
printf:  "=====\\n";
printf{k in K}:  "Hospital [%2d]:  %.2f\\t%.2f\\n", k,
sum{h in H}Q[h,k], sum{h in H, i in I, j in J} x[h,i,j,k];
```

MathProgramming - Printing a Report

```
printf: "=====\n";
printf: "PHC : Capacity\t Met\tUse(%%)\n";
printf: "=====\n";
printf{i in I}: "PHC [%2d]: %.2f\t%.2f\t%.2f%%\n", i,
sum{h in H}Y[h,i], sum{h in H, j in J, k in K} x[h,i,j,k],
((sum{h in H, j in J, k in K} x[h,i,j,k])/(sum{h in H}Y[h,i]))*100;
printf: "=====\n";
printf: "IHC :\tFlow\tUse(%%)\n";
printf: "=====\n";
printf{j in J: z[j] >= 1}: "IHC [%2d]:\t%.2f\t%.2f%%\n",
j, sum{h in H, i in I, k in K} x[h,i,j,k],
sum{h in H, i in I, k in K} x[h,i,j,k]/(W[j]*z[j])*100;
```

MathProgramming - Printing a Report

```
printf: "=====\n";
printf: "Detailed Flow\n";
printf: "=====\n";
printf: "Pac\tPHC\tIHC\tHosp.\tFlow\n";
printf: "=====\n";
printf{h in H, i in I, j in J, k in K: x[h,i,j,k] > 0}:
 "[%d]\t[%d]\t[%d]\t[%d]:\t%.2f\n", h,i,j,k,x[h,i,j,k];
printf: "=====\n";
```

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